INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Pembrolizumab + pemetrexed + cisplatin

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00569
Version 5

> Induction: all three medicines on Day 1 every 21 days for up to four cycles. Maintenance: cisplatin stops; pembrolizumab and pemetrexed may continue every 21 days while appropriate. The team agrees duration and review points.

Treatment stages

01 **Induction · up to four cycles**
Three medicines

02 **Maintenance · after induction**
Two medicines

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Pembrolizumab + pemetrexed + cisplatin
National Cancer Control Programme (NCCP) 00569

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Use the oncology emergency instructions for fever, shivers or suddenly feeling unwell, even with a normal temperature. New cough/breathlessness, severe diarrhoea, reduced urine, confusion or severe rash need prompt advice. Severe breathing difficulty, chest pain, collapse or major bleeding need emergency help. Immune symptoms may start after treatment stops.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

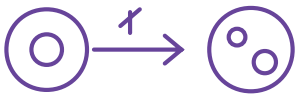
Three medicines. Different jobs.

This combination aims to control metastatic non-squamous lung cancer, help symptoms and extend life. Your team weighs likely benefit, risks and what matters to you.

IMMUNOTHERAPY

Pembrolizumab

Keytruda



Releases an immune brake

PD-1 is a control switch on T cells, a type of immune cell. When it connects with PD-L1 or PD-L2, the T cell receives a signal to reduce its activity. Some cancers use this pathway to escape immune attack. Pembrolizumab is an antibody that blocks PD-1, helping T cells recognise and attack the cancer. It does not directly poison cancer cells like chemotherapy. Releasing this brake can also allow immune cells to attack healthy organs, which explains the different side-effect pattern and why symptoms can appear after treatment stops.

Immune cell → Brake blocked → Cancer cell

What to watch for

The immune system can also inflame healthy organs. New cough, diarrhoea, unusual weakness, severe headache, thirst or a spreading rash need prompt advice. Effects can start during treatment or after it finishes.

CHEMOTHERAPY

Pemetrexed

Alimta



Disrupts deoxyribonucleic acid (the cell's genetic material) (DNA) building

Pemetrexed is an antifolate: it blocks several enzymes that cells use to turn folate-dependent materials into DNA and RNA building blocks. With fewer building blocks, cancer cells have more difficulty copying their genetic instructions and dividing. Healthy rapidly renewing tissues, including bone marrow and the mouth lining, can also be affected. That helps explain low blood counts and mouth soreness. Prescribed folic acid and vitamin B12 reduce toxicity; they are part of the treatment, not supplements to omit because the drug is an antifolate.

DNA building blocks → Supply disrupted → Cell division slows

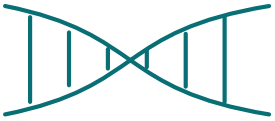
What to watch for

Healthy tissues can be affected too: low blood counts, a sore mouth, rash and kidney problems are important. Folic acid, vitamin B12 and prescribed steroids help reduce particular risks.

CHEMOTHERAPY

Cisplatin

Platinum chemotherapy



Damages cancer-cell DNA

Cisplatin forms links in DNA that disrupt copying and repair, helping kill cancer cells. Healthy tissues can also be affected. Kidney, hearing and nerve effects are important; blood tests, symptom checks and prescribed hydration help the team manage risk.

DNA strands → Platinum links → Copying disrupted

What to watch for

Report reduced urine, ringing in the ears, hearing changes or tingling before the next treatment. Tell the nurse immediately about swelling or pain around the drip, rash, chest tightness or feeling suddenly unwell.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding your diagnosis and treatment aim

Start here if the names and abbreviations are new. You can read one section at a time and return when you need it.

What do the cancer terms mean?

Non-small-cell lung cancer (NSCLC) is a broad group of lung cancers. “Non-squamous” describes the cells seen by the pathologist and includes adenocarcinoma. “Metastatic” means the cancer has spread beyond its original site. “First-line” means the first drug treatment chosen for this stage of illness. A regimen is a planned combination and schedule of medicines.

What is the treatment trying to achieve?

The usual aims in this setting are to control cancer, help symptoms and extend life while preserving quality of life. It is generally not given with an expectation of cure. Some people have prolonged benefit; others do not respond. Ask what success would look like for you: easier breathing, less pain, stable scans, more time or a balance of these.

Why are tumour tests important?

The team considers the cancer’s molecular results and PD-L1 result alongside your health. EGFR and ALK alterations can point towards a different treatment approach. PD-L1 is a protein involved in immune signalling; its result helps inform choices but is not a guarantee of benefit. This combination has evidence across PD-L1 groups, including low expression. Ask whether all relevant tumour testing is complete before your plan is finalised.

What are the alternatives?

The right comparison depends on tumour tests, symptoms, fitness and your preferences. Ask whether targeted therapy, immunotherapy alone, a different chemotherapy approach, a clinical trial or symptom-focused care is relevant. Supportive and palliative care can accompany active cancer treatment: help with breathlessness, pain, appetite and family concerns is not reserved for the last days of life.

Why this treatment is offered

Adding pembrolizumab to chemotherapy improved survival and delayed progression in the trial population. Results cannot predict one person’s benefit.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Induction → review → maintenance

Treatment stages

01 **Induction · up to four cycles**
Three medicines

02 **Maintenance · after induction**
Two medicines

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

THREE MEDICINES
Induction · up to four cycles

C1 C2 C3 C4

Induction: all three medicines on Day 1 every 21 days for up to four cycles.

TWO MEDICINES
Maintenance · after induction

M1 M2 M3 …

Maintenance: cisplatin stops; pembrolizumab and pemetrexed may continue every 21 days while appropriate. The team agrees duration and review points.

Maintenance means ongoing treatment, not treatment without side effects. Your plan can change after discussion.

BASELINE → TREATMENT → FOLLOW-UP

Treatment checks across the course

Relative stages, not equally spaced dates. No patient dates are needed. Agree who arranges and reviews each check.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00569 v5 · official regimen

THE WHOLE TREATMENT JOURNEY

Pembrolizumab + pemetrexed + cisplatin: treatment and checks

First-line metastatic non-squamous non-small-cell lung cancer (NSCLC) without EGFR or ALK alterations

Induction: all three medicines on Day 1 every 21 days for up to four cycles. Maintenance: cisplatin stops; pembrolizumab and pemetrexed may continue every 21 days while appropriate. The team agrees duration and review points.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First induction dose	During induction	Maintenance review	After treatment
Treatment NCCP 00569	Confirm indication and planned phases	Three medicines on Day 1	Every 21 days; up to four induction cycles	Continue pembrolizumab + pemetrexed every 21 days while appropriate	Agree follow-up and next decisions
Bloods and hormones NCCP 00569	full blood count (FBC), kidney/liver, glucose, thyroid and hepatitis B/C screening	Review results and baseline symptoms	FBC, kidney/liver and glucose each cycle; thyroid every 3–6 weeks	Continue cycle bloods and thyroid checks; review symptoms	Follow unresolved abnormalities and the team's plan
Nerves, hearing and kidneys NCCP 00569	Document baseline nerves/hearing; renal assessment	Check cisplatin hydration	Nerve/hearing review before platinum; report new symptoms	Platinum stops; continue follow-up of any ongoing toxicity	Persistent symptoms need review
Support medicines NCCP 00569	Folate/B12 lead-in and reconciled steroids	Use the induction support prescription	Check adherence, effectiveness and side effects	Continue folate/B12/rash prophylaxis; revise antiemetics	Folate 21 days after last pemetrexed
Immune effects and cancer review NCCP 00569	Explain alert card, symptoms and treatment aims	Report infusion symptoms immediately	Report new cough, diarrhoea, rash or endocrine symptoms between visits	Continue pembrolizumab while appropriate; scans per consultant plan	Delayed immune effects remain possible

What each check is for

1 Before treatment

Before first dose

Establish suitability and baseline

Ask / act: Confirm results, support prescriptions and responsible teams.

2 **Phase change**

After up to four induction cycles

Active medicines and support change

Ask / **act:** Confirm maintenance medicines and revised support; continue immune monitoring.

3 **After final treatment**

Completion or treatment change

Delayed effects and continuing support

Ask / **act:** Give the follow-up and urgent-contact plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Appointments and time in the unit

Pembrolizumab first over 30 minutes, then pemetrexed over 10 minutes. Cisplatin starts 30 minutes after pemetrexed and runs over 2 hours; pre/post hydration adds time during induction. Clinic time includes assessment, preparation and waiting; ask your unit about the full visit.

STEP 2

Treatment phase

STEP 3

Medicines and practical help

Bring the hydration, anti-sickness, folate, B12 and steroid plan. Ask who organises B12 and blood tests; check fluid advice for your own health. Keep the immunotherapy alert card and urgent numbers with you.

STEP 4

After treatment

Response imaging follows the consultant plan; no fixed universal scan interval is specified in these NCCP regimens. Immune toxicity can arise after the final pembrolizumab dose. Continue symptom vigilance, immunotherapy alert card and the team's follow-up plan. Continue folic acid during pemetrexed and for 21 days after its last dose; B12 every three cycles while pemetrexed remains active.

Before the first infusion

Use this as a conversation checklist. Your own hospital's instructions and prescription take priority.

Tell the team your starting point

Describe your usual breathing, bowel pattern, energy, skin symptoms and ability to manage daily tasks. Mention autoimmune illness, organ transplant, previous lung inflammation, infections and all regular medicines. Changes are easier to recognise when the team knows what is normal for you. Do not minimise symptoms because you are worried treatment will be delayed.

Make the first week manageable

Have the oncology and out-of-hours numbers available, a working thermometer, your medicine schedule and a way to get urgent help. Ask who to ring if you miss a supportive medicine. Plan transport and practical help if needed. Bring your questions and, if you wish, someone you trust to listen and take notes.

During the infusion visit

Expect a symptom review and checks before drugs are released. Treatment is given through a cannula or another agreed venous access device. Tell the nurse immediately about burning or swelling around the drip, itching, flushing, dizziness or breathing changes. Waiting, pharmacy preparation and observation add to the time spent receiving the medicines.

What your tests and scans are checking

Different checks answer different questions: whether treatment is safe today, whether a symptom needs investigation, and whether the cancer is responding.

Blood counts: three separate jobs

Neutrophils help fight infection; a low level can make infection dangerous. Haemoglobin carries oxygen; a low level can contribute to tiredness and breathlessness. Platelets help stop bleeding; low levels can cause bruising or bleeding. Feeling well does not prove your counts are normal, and a previous normal result does not rule out a new problem.

Kidney, liver and hormone checks

Kidney tests help assess whether cisplatin and pemetrexed are safe; cisplatin also needs a prescribed hydration plan. Liver tests can show injury before obvious symptoms develop. Pembrolizumab can disturb hormone-producing glands, so blood glucose and thyroid monitoring matter too. Symptoms can prompt additional tests. Always report how you feel: routine blood tests alone do not exclude every immune side effect.

Scans and the treatment decision

Your team sets the scan schedule and compares scans with symptoms and examination findings. Stable disease can be a worthwhile result. New symptoms need review even before the next scan. An unusual scan pattern sometimes needs clarification; do not assume that apparent growth is simply an immune response or that feeling worse means treatment is working.

Why a delay or a change can be appropriate

A pause may allow recovery or investigation. Chemotherapy doses can sometimes be reduced. Pembrolizumab is generally held or stopped for relevant toxicity rather than dose-reduced. Ask which drug is changing and why; stopping one drug does not automatically mean stopping every part of the plan. Long-term treatment decisions belong in a discussion about benefit, risk and what matters to you.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Treatment	 **Day 1** Day 1 · infusion visit
Nausea / appetite	 **Days 1–5** Early days · may continue longer
Blood counts / infection	 **Days 1–21** Counts may fall between visits · timing varies
Energy / mouth / bowels	 **Days 1–21** Symptoms vary across the cycle
Immune effects	 **Days 1–21** Any day · also after treatment ends

Induction and maintenance differ: Induction: all three medicines on Day 1 every 21 days for up to four cycles. Maintenance: cisplatin stops; pembrolizumab and pemetrexed may continue every 21 days while appropriate. The team agrees duration and review points.

After the final infusion: Response imaging follows the consultant plan; no fixed universal scan interval is specified in these NCCP regimens. Immune toxicity can arise after the final pembrolizumab dose. Continue symptom vigilance, immunotherapy alert card and the team's follow-up plan. Continue folic acid during pemetrexed and for 21 days after its last dose; B12 every three cycles while pemetrexed remains active.

Symptoms do not follow a fixed calendar: Effects may overlap between cycles. Immune symptoms can occur after treatment stops. The diagram cannot identify a safe day or predict recovery.

> **Why these bands stay broad:** The treatment bands show a reference 21-day cycle. Induction and maintenance contain different medicines. Symptom bands are broad educational illustrations, not measured daily risks or a prediction of when you

will feel well; immune-related effects can occur after treatment ends. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.

There are no guaranteed "safe days". Use the oncology emergency instructions for fever, shivers or suddenly feeling unwell, even with a normal temperature. New cough/breathlessness, severe diarrhoea, reduced urine, confusion or severe rash need prompt advice. Severe breathing difficulty, chest pain, collapse or major bleeding need emergency help. Immune symptoms may start after treatment stops.

Between-treatment notes

Day 1: Your infusion day

Your team checks symptoms and blood results before treatment. Tell your nurse immediately about pain at the drip site, itching, flushing or breathing changes. The time in hospital includes checks and preparation, not just infusion time. Before leaving, check your take-home medicines, contact details and next appointment.

Day 2–5: The first days at home

Feeling sick can start on treatment day or later. Appetite, taste, sleep or bowel habits may change. Take prescribed supportive medicines as directed; ask for help if they are not controlling symptoms. Contact the team if vomiting persists, you cannot keep fluids down, or you develop new diarrhoea. Do not assume diarrhoea is ordinary chemotherapy upset.

Day 6–14: Keep checking how you feel

Blood counts may be reduced between treatments. You cannot tell your count from the date or from how well you feel. Fatigue or mouth symptoms may affect eating and daily activity. Use the urgent advice below for fever, shivering or suddenly feeling unwell, including with a normal temperature.

Day 15–21: Prepare for your next review

Some symptoms may settle; others can continue or appear for the first time. Improvement does not prove your blood counts have recovered. The next treatment depends on your review and tests. Bring your symptom record and questions. Confirm the next appointment: the next Day 1 is usually 21 days after this cycle's Day 1, if treatment proceeds.

Support around the medicines

1 Induction support

You receive anti-sickness medicines and a fluid plan around cisplatin. Take prescribed folic acid, B12 injections and rash-prevention steroids. Ask for one clear steroid schedule.

2 Maintenance support

Folic acid, B12 and rash-prevention medicines continue with pemetrexed; cisplatin fluids and some anti-sickness medicines may stop. Immune symptom vigilance continues.

Side effects: what they mean and what to do

These are selected possibilities, not a prediction or a complete medicine leaflet. You do not need to work out the cause before asking for help.

Nausea, vomiting and appetite change

Medicines can stimulate sickness pathways in the gut and brain. Take preventive anti-sickness medicines as prescribed, even when you initially feel well. Smaller meals and frequent sips may be easier. Report poor control early so the plan can be adjusted. Persistent vomiting, inability to keep fluids down or much less urine need prompt advice. Do not wait until you are very dehydrated.

Fatigue: common, but not always “just treatment”

Fatigue may reflect treatment, disturbed sleep, reduced intake, anaemia or another illness. Balance rest with manageable movement and prioritise activities that matter to you. A sudden change, severe weakness, dizziness, breathlessness or inability to manage basic tasks needs assessment; hormone problems and other complications can resemble ordinary tiredness. Tell the team how fatigue affects daily life, not just that you feel tired.

Mouth soreness, ulcers and taste

Treatment can injure the lining of the mouth. Use a soft toothbrush, gentle mouth care and the rinses recommended by your team. Choose softer foods and avoid foods that sting. Ask for pain relief early. Report ulcers, white patches, painful swallowing or reduced intake; difficulty drinking or fever needs urgent advice. Arrange dental care with the oncology team before procedures.

Diarrhoea or abdominal pain

Pemetrexed can affect the bowel, but pembrolizumab can also cause bowel inflammation (colitis). Report new diarrhoea promptly, including how many stools above your normal pattern, blood or mucus, night-time symptoms and pain. Do not assume it is a harmless expected effect or rely on anti-diarrhoeal medicine without the team’s instructions. Severe pain, bleeding or dehydration needs urgent assessment.

Constipation

Anti-sickness drugs, opioid painkillers, less food and less movement can slow the bowel. Ask about a bowel plan if you are prone to constipation or take opioids. Keep track of your usual pattern. Increasing abdominal pain, vomiting, a swollen abdomen or inability to pass wind needs prompt assessment rather than simply increasing laxatives.

Bruising, bleeding or breathlessness

Chemotherapy may reduce platelets or red cells. Report unexpected bruises, nosebleeds, bleeding gums or increasing breathlessness. Heavy bleeding, black stools with weakness, severe chest pain or severe breathing difficulty need emergency help. Do not start iron or alter blood thinners without review: the cause and treatment need to be established.

Skin, hair, nerves and hearing

Skin can become dry, itchy or develop a rash; report new changes, especially if spreading. Hair may thin. Tingling, numbness or hearing changes should be reported before the next dose because they may affect the plan. Blisters, peeling skin or sores involving the eyes or mouth need urgent assessment. Use gentle skin care and discuss suitable sun protection.

Pembrolizumab: symptoms that deserve special attention

Immune treatment can cause inflammation in healthy organs, during treatment or after it ends. Keep your immunotherapy alert card and tell other healthcare professionals you have received pembrolizumab.

Lungs, bowel, liver and kidneys

New cough or breathlessness, diarrhoea, yellow skin or eyes, dark urine, reduced urine or swelling can signal inflammation. These symptoms also have other causes, including infection and cancer. Contact the team promptly for assessment rather than trying to identify the organ or diagnosis yourself.

Hormones, heart and nervous system

Report unusual weakness, persistent headache, dizziness, marked thirst, frequent urination, palpitations, muscle weakness or changes in vision. Severe chest pain, collapse, confusion, seizures or major breathing difficulty require emergency care. Some hormone effects need ongoing replacement treatment even after immunotherapy stops.

Why early reporting matters

The team may pause treatment and investigate. Some reactions need steroids or other immune-suppressing treatment; others need hormone replacement. These are different from the short planned steroid course around chemotherapy. Do not start leftover steroids, change their dose or stop a prescribed course yourself. Carry the treatment name when attending emergency care.

Living with treatment

The aim is a plan that fits your life as well as your treatment. Bring practical concerns to the team early.



Food, fluids and weight

Aim for enough food and fluid to maintain strength, using foods you tolerate. Tell the team about weight loss, swallowing problems or difficulty shopping or cooking; ask for dietitian support. A fixed fluid target is not right for everyone, particularly with heart or kidney disease. “Anti-cancer” diets, fasting and high-dose supplements are not substitutes for treatment and can create nutritional or interaction problems.

Movement, work and rest

Try activity that feels manageable and increase or reduce it according to symptoms. Short walks or simple daily tasks may be more realistic than your usual exercise. Discuss work adjustments and travel plans. Do not drive if dizzy, unusually tired or affected by sedating medicines. Sudden deterioration should prompt assessment, not an attempt to push through exercise.

Infection precautions without isolation

Wash hands, practise food hygiene and avoid close contact with people who are unwell. Ask about vaccinations and their timing; some vaccines are unsuitable during treatment. You can usually still spend time with family. Keep your urgent contact plan available: precautions reduce risk but do not make fever or sudden illness safe to watch at home.

Intimacy, fertility and pregnancy

Discuss fertility preservation before treatment if relevant. Treatment may harm a developing baby; ask for a written contraception plan covering all three medicines and the period after the last dose. Do not breastfeed during treatment; confirm when it would be safe to restart. Tell the team promptly about a possible pregnancy. Ask about barrier protection and handling body fluids under your hospital's advice.

Emotional support and choices

It is reasonable to feel worried, angry, uncertain or overloaded. Ask for psycho-oncology, counselling, social work or family support. You can request information in smaller pieces, an interpreter or a written summary. Tell the team what you most want to preserve in daily life. You can revisit treatment decisions and ask for more time or another explanation.

A short treatment dictionary

Terms you may hear in clinic.

Cycle · induction · maintenance

A cycle is a planned treatment-and-interval period. Induction is the initial combination phase. Maintenance is ongoing treatment after that phase; it does not mean side-effect-free treatment.

Response · stable disease · progression

Response means the cancer has reduced by the assessment used. Stable disease means it has not changed enough to meet response or progression criteria. Progression means growth or spread meeting the assessment criteria. Your team interprets these alongside how you are feeling.

Neutropenia · anaemia · thrombocytopenia

Neutropenia means low infection-fighting neutrophils. Anaemia means low haemoglobin. Thrombocytopenia means low platelets. These are different results with different implications; one normal number does not establish that all treatment risks are low.

Immune-related · colitis · pneumonitis

Immune-related effects come from immune activity against healthy tissue. Colitis means inflammation of the colon; pneumonitis means inflammation in the lungs. A symptom alone cannot establish either diagnosis.

Cisplatin: hydration, hearing and nerves

These checks belong to the cisplatin phase. Problems that continue after it stops still need review.

Fluids and electrolytes

You receive prescribed fluids around cisplatin. The amount and timing depend on your health and local policy; do not force a fixed fluid target if you have heart or kidney disease. Report reduced urine, swelling, dizziness or persistent vomiting.

Hearing and nerve changes

Tell the team about ringing in the ears, changed hearing, numbness or difficulty with fine movements before the next dose. Some effects can last; early reporting helps guide treatment decisions.

Questions worth asking

How will we know if it is helping?

Ask your oncologist what the treatment is aiming to achieve for you, when scans are planned and what would lead to continuing, changing or stopping it. How you feel and the amount of side effects do not reliably show how well treatment is working.

Why might a cycle be delayed?

A delay can give time to recover or investigate a new symptom. It is a safety decision, not a personal failure. Ask which medicines are affected, what needs to improve and when you will be reviewed.

Will I get every side effect?

No. The body map shows selected possibilities, not a prediction. Ask about common effects, serious less common effects, and any long-term risks that matter particularly to you. The same symptom can have several causes.

What should I organise before starting?

Confirm blood-test and Day Ward arrangements, transport home, your medicines schedule and a thermometer. Keep the oncology contact instructions and immunotherapy alert card available. Ask for an interpreter or accessible information if useful.

What else belongs in the consent conversation?

Your priorities, expected benefit, alternatives including supportive care, fertility, contraception, pregnancy or breastfeeding, work, driving and the possibility of lasting side effects. You can ask more questions or reconsider treatment at any stage.

Do more side effects mean the treatment is working better?

No. Side effects are not a reliable personal measure of cancer response. Report them so they can be treated; do not tolerate serious symptoms in the hope that they are a sign of benefit.

What should I say when calling about a symptom?

Give the current medicines or say you are in maintenance, the last treatment date, what changed and when, your temperature and whether you can eat, drink and pass urine. Do not delay calling to collect every detail.

Try explaining the plan back: Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

Pembrolizumab

- Immune inflammation of lungs, bowel, liver, kidneys, glands, skin and other organs
- Serious infusion reactions
- Effects can develop after treatment ends

Pemetrexed

- Low blood counts and infection
- Fatigue, nausea, mouth soreness and rash
- Kidney toxicity; folate and B12 supplementation are required

Cisplatin

- Nausea and vomiting
- Kidney injury and electrolyte changes
- Hearing loss or tinnitus
- Numbness/tingling and other nerve effects

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Induction: cisplatin-containing cycles 1–4

Active medicines: pembrolizumab + pemetrexed + cisplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before treatment Day 1, then daily Days 2–3

Medicine	Dose / route	When
Ondansetron	16 mg orally	Day 1 Individualise for QT risk and patient factors.
Antiemetic dexamethasone reference	12 mg orally Day 1; 8 mg orally daily Days 2–4	NCCP NCIS V8 high-risk general row Reconcile with pemetrexed rash steroid and aprepitant interaction; issue one clear prescription, do not add overlapping courses.
Olanzapine	5 mg orally daily	Days 1–4 Cisplatin high-risk general HEC row. Sedation/metabolic cautions; consultant may individualise.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS table does not specify an end day here; prescribe duration and suitability. Concomitant olanzapine increases extrapyramidal adverse-effect risk.
Folic acid	350–1000 micrograms orally daily	At least five doses in seven days before first pemetrexed; throughout treatment and 21 days after the final dose
Vitamin B12 (hydroxocobalamin)	1000 micrograms intramuscular (IM)	Week before first pemetrexed and every three cycles thereafter Later doses may be on the pemetrexed day.
Pemetrexed rash prophylaxis: dexamethasone equivalent	4 mg orally twice daily	Day before, day of and day after pemetrexed This is the regimen rash schedule; prescriber/pharmacy must reconcile it with antiemetic dexamethasone into one unambiguous prescription, including aprepitant interaction. Do not simply add both courses.
Cisplatin hydration and electrolytes	Local individual prescription	Before and after cisplatin NCCP suggested framework: pre 1 L sodium chloride 0.9% over 60 minutes; post 1 L over 2 hours with 10 mmol magnesium sulfate and 20 mmol potassium chloride. Local policy and fluid/electrolyte status govern. Mannitol optional; routine loop diuretic is not required unless fluid overload.

Additional phase-specific support

- Cisplatin requires high-risk antiemetics and individual hydration/electrolyte review.
- Folic acid and B12 must be in place before pemetrexed and continue into maintenance.
- Combine rash and antiemetic dexamethasone directions into one reconciled prescription; do not stack courses.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Maintenance: pembrolizumab + pemetrexed

Active medicines: pembrolizumab + pemetrexed

Emetogenic risk: low

Medicine	Dose / route	When
Folic acid	350–1000 micrograms orally daily	At least five doses in seven days before first pemetrexed; throughout treatment and 21 days after the final dose
Vitamin B12 (hydroxocobalamin)	1000 micrograms intramuscular (IM)	Week before first pemetrexed and every three cycles thereafter Later doses may be on the pemetrexed day.
Pemetrexed rash prophylaxis: dexamethasone equivalent	4 mg orally twice daily	Day before, day of and day after pemetrexed This is the regimen rash schedule; prescriber/pharmacy must reconcile it with antiemetic dexamethasone into one unambiguous prescription, including aprepitant interaction. Do not simply add both courses.
Low-risk antiemetic / rescue plan	Individual prescription	Around pemetrexed Account for the existing rash-prophylaxis steroid. Do not automatically continue induction aprepitant/olanzapine/high-risk steroid course.

Additional phase-specific support

- Continue daily folate, B12 every three cycles and pemetrexed rash prophylaxis.
- Cisplatin and its hydration protocol end after induction; reassess supportive prescriptions.
- Keep immune, renal, glucose and thyroid monitoring.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Pembrolizumab + pemetrexed + cisplatin

Induction: all three medicines on Day 1 every 21 days for up to four cycles. Maintenance: cisplatin stops; pembrolizumab and pemetrexed may continue every 21 days while appropriate. The team agrees duration and review points.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hormones / brain

Pembrolizumab

Unusual weakness, dizziness, headache, thirst or frequent urination: report promptly.

2 Lungs

Pembrolizumab

New cough or breathlessness needs prompt assessment; infection, cancer or clot can look similar.

3 Heart

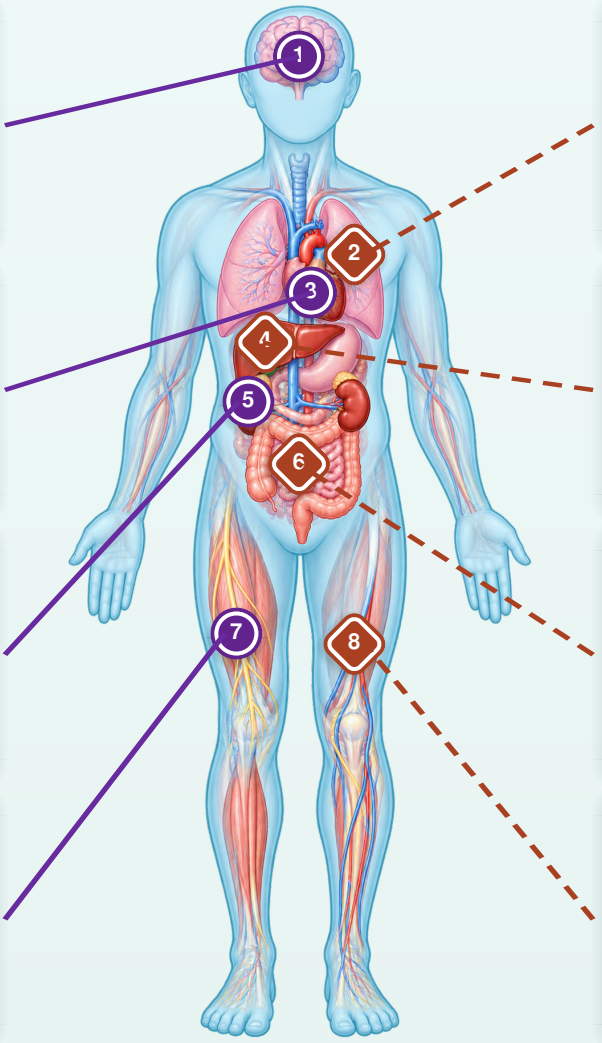
Pembrolizumab

Chest pain, racing heartbeat or fainting needs urgent assessment.

4 Liver

Pembrolizumab / pemetrexed

Yellow skin/eyes or dark urine; tests also check changes before symptoms.



5 Kidneys

Pembrolizumab / cisplatin / pemetrexed

Report much less urine, swelling or persistent vomiting; kidney results affect treatment.

6 Bowel / mouth

Pembrolizumab / chemotherapy

New diarrhoea, severe tummy pain, mouth ulcers or inability to drink need advice.

7 Skin / nerves

Pembrolizumab / cisplatin

Report rash, blisters, numbness, weakness or problems with balance or hand function.

8 Blood / infection

pemetrexed / cisplatin

Fever, shivers or suddenly feeling unwell: contact oncology immediately.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Use the oncology emergency instructions for fever, shivers or suddenly feeling unwell, even with a normal temperature. New cough/ breathlessness, severe diarrhoea, reduced urine, confusion or severe rash need prompt advice. Severe breathing difficulty, chest pain, collapse or major bleeding need emergency help. Immune symptoms may start after treatment stops.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00569 v5 · pNCCP 00569 treatment and supportive-care sections; NCIS V8**
   https://healthservice.hse.ie/documents/6890/569_V5_Pembrolizumab_PEMEtrexed_and_CISplatin.pdf
2. **NCCP NCIS Medical Oncology antiemetics V8**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
3. **KEYNOTE-189 five-year published follow-up**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC10082311/
4. **HSE · chemotherapy safety and when to contact your team**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/